SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Pegylated liposomal doxorubicin · every 4 weeks

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Source-defined advanced breast, ovarian, endometrial cancer or soft-tissue sarcoma · NCCP 00205 · version 8 · 28-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
The aim is to control the cancer and its symptoms. The team reviews benefit, side effects and your priorities before continuing.

Schedule
Pegylated liposomal doxorubicin on Day 1 every 28 days, typically for six cycles or until progression or unacceptable toxicity. The team confirms the intended duration and reviews benefit.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Pegylated liposomal doxorubicin
LIPOSOMAL CHEMOTHERAPY

- ☐ Painful red palms / soles (hand-foot syndrome)
- ☐ Mouth soreness or ulcers affecting eating and drinking
- ☐ Low blood counts, infection, fatigue and nausea
- ☐ Heart damage and infusion reactions; risk depends partly on previous treatment

Discuss the exact indication, expected benefit, alternatives, uncertainty, serious and lasting risks, contraception and fertility, venous access, monitoring and the urgent contact plan. This is the pegylated liposomal formulation (for example Caelyx), not conventional doxorubicin or non-pegylated Myocet. These formulations and doses must not be interchanged.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00205 v8.

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